

YOURSTORY



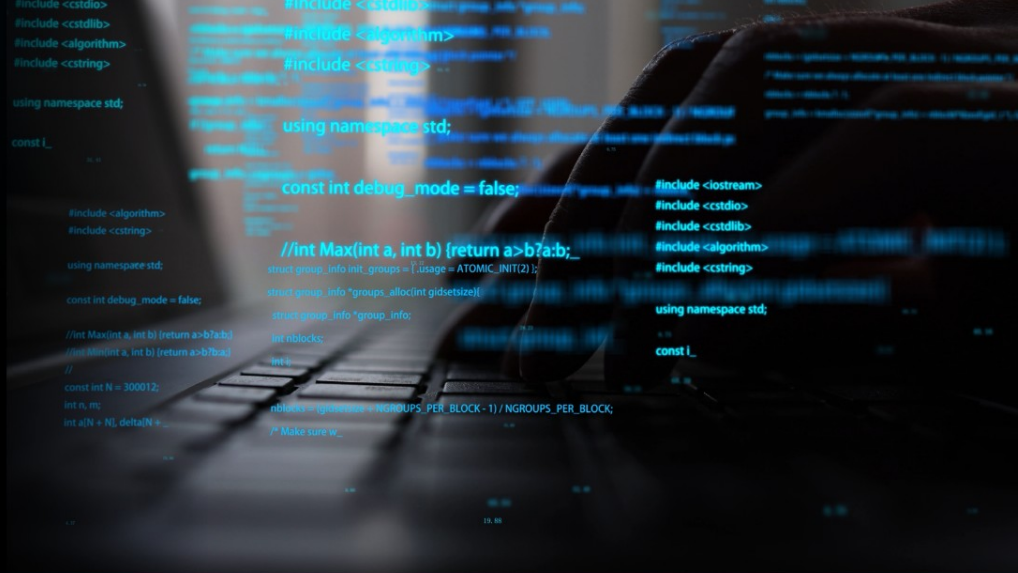
H2S



Coco CLI

HACKATHON

GCC Edition



Team Name : **Docket Rocket**

Team Leader Name : **Nissy Niharika Hanamkonda**

Team Size : **1**

Problem Statement : **Patient and Member 360 and Clinical or Regulatory Document Copilot**

Docket · every AI claim goes on trial. If a record doesn't back it up, it is struck.

1 · PROBLEM BRIEF

Reviewers can't act on an AI answer they can't check

Business problem

Health plans decide prior authorizations and care-management enrollment from records split across EHR, medical claims, pharmacy claims and PDFs. An AI answer without a source can't be used in a decision.

Target users

Utilization-management nurses and pharmacists, care managers, and compliance and appeals teams at health plans (payers).

Pain point → improvement

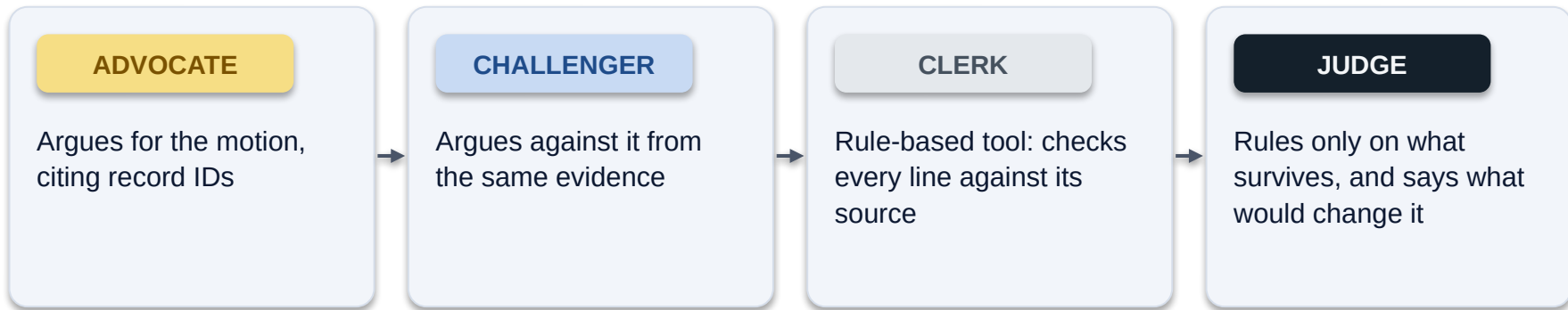
Today reviewers search systems and policies by hand, and contradictions between records get missed. Docket cites or strikes every statement, flags conflicts and drafts the specific reason.

Industry context

CMS-0057-F, in effect since Jan 2026, requires prior-auth decisions within 7 days (72 hours if urgent) and a specific reason for every denial, for Medicare Advantage, Medicaid, CHIP and federal exchange plans.

SOLUTION

Docket tries every question like a court case



Possible rulings



The Judge never denies. Adverse decisions go to a licensed reviewer with a drafted specific reason.

Never an opaque prediction: risk uses the published LACE readmission index, where every point traces to a record.

2 · ARCHITECTURE DIAGRAM

Built end to end inside Snowflake with CoCo



WORKING DEMO

Prototype: three cases, three different rulings

PRIOR AUTHORIZATION · PHARMACY · M-20417-02

Motion to approve prior authorization PA-26-10021 for sacubitril/valsartan 24/26 mg twice daily

Received 2 Oct 2026 · standard request Decision due **9 Oct 2026** · 7 calendar days (CMS-0057-F, standard) Governing text **POL-RX-HF-12**

Member **Lorraine Pembury** (M-20417)

EHR

Claims

Pharmacy

Notes

Policy & label

Query

▶ **Replay trial**

CLERK

Opens the record

1

Docket M-20417-02. Standard prior authorization received 2 Oct 2026; decision due 9 Oct 2026. Criteria come from plan policy RX-HF-12 and the US prescribing information.

ADVOCATE

Argues for approval

2

Criterion 2(a): the echocardiogram on 16 Sep 2026 shows an LVEF of 30%, within the 12-month window. **OBS-ECHO-0916** **POL-RX-HF-12**

3

Criterion 2(b): the cardiology note documents NYHA class III symptoms. **DOC-CARD-0930**

4

Criterion 2(c): the prescriber is a cardiologist. **PA-26-10021**

5

~~The member has tolerated ARB therapy in the past.~~
STRUCK - HEARSAY No exhibit cited. The Clerk queried 36 months of pharmacy claims and found no ARB fills. Line struck.

CHALLENGER

Argues against approval

6

Her ejection fraction is 45%, which is not reduced. **OBS-ECHO-2303**
STALE Exhibit dated 11 Mar 2023 falls outside the 12-month window in §2(a) and is superseded by OBS-ECHO-0916. Given no weight.

7

Lisinopril, an ACE inhibitor, was filled on 26 Sep, six days after the discharge order to stop it. The label bars concomitant use and requires a 36-hour gap.

RULING

Safety hold · route to clinical pharmacist

Clinical criteria are met. Approval is blocked by two open conflicts that touch label contraindications. Only a licensed reviewer can deny.

1

LVEF 30% within 12 months, NYHA III, and a cardiology prescriber meet §2(a)–(c).
OBS-ECHO-0916 **DOC-CARD-0930**
PA-26-10021

2

An active lisinopril fill conflicts with the 36-hour washout in §4 and the label. **RX-LIS-0926**
LBL-SV-4

3

A possible ACE-inhibitor angioedema in 2024 is undocumented in the allergy list. **DOC-ED-241103**
ALG-LIST

4

Potassium 5.3 mmol/L needs a monitoring plan if approved. **OBS-K-0919** **LBL-SV-5**

CLERK'S TALLY

6

admitted

1

struck

1

stale

2

conflicts

Care management **Granted**

LACE 14 vs threshold 10; new diuretic dose unfilled 15 days

Heart-failure drug PA **Safety hold**

Criteria met, but 2 record conflicts touch label contraindications

Lumbar MRI PA **Request records**

23 of 42 therapy days; “progressive” claim struck

All data synthetic. Click any citation in the demo to open the exact source row or passage.

3 · IMPACT STATEMENT

Every decision traceable, and faster to defend

22

statements checked across the 3 demo cases

3

struck for missing or unsupported sources

2

safety conflicts found that a criteria-only check would miss

100%

of surviving statements cite a record

Measurable outcomes (pilot)

- Reviewer minutes per prior auth
- Share of decisions inside the 7-day clock
- Denials overturned on appeal
- Answers fully cited (target 100%)

Scalability

- Runs inside Snowflake, next to governed data
- Streams re-try only members with new records
- Dynamic tables refresh incrementally
- One skill reused across any policy set

Beyond the demo

- Drug-safety alerts when a label changes
- Appeal packets with cited evidence
- Quality-gap evidence for care teams
- Provider-side chart contradiction audits

Demo figures come from the synthetic prototype; pilot outcomes are what we would measure, not results.

ADDITIONAL · COCO EVIDENCE

CoCo across the full lifecycle

Planning

Profiled the data; wrote the ontology and design doc

Development

Synthetic data, Member 360 views, LACE, Cortex Search, Clerk UDF, cited-answer function

Execution

Stream + nightly task at 02:00 IST that re-tries changed members

Testing

Golden questions; a fake citation must be struck; LACE must equal 14

Reusable skill: evidence-courtroom, so other teams can point it at any policy and evidence set. [Add CoCo screenshots here.]

Additional Slide

ADDITIONAL · TRUST AND SAFETY

Guardrails: the Clerk's four objections

Objection	Test	Effect
Hearsay	The statement cites no record	Struck
Not supported	The cited record doesn't contain the claimed fact	Struck
Stale	Evidence is outside the policy's time window	No weight
Conflict	Two records disagree about the same fact	Blocks approval

Numbers and dates must match the source exactly; wording is checked with an entailment prompt. When nothing answers a question, Docket says so instead of guessing, and it refuses to give treatment instructions.

Additional Slide

YOURSTORY



1125

Coco CLI HACKATHON

GCC Edition

THANK YOU



```
#include <algorithm>
#include <string>

using namespace std;

const int N = 300012;
int n, m;
int a[N + N], delta[N + N];

//int Max(int a, int b) {return a > b ? a : b;}
//int Min(int a, int b) {return a < b ? a : b;}

// Make sure w
```